

WORKFLOW FOR ALLEGED SEXUAL ASSAULT IN THE CHILDREN'S EMERGENCY

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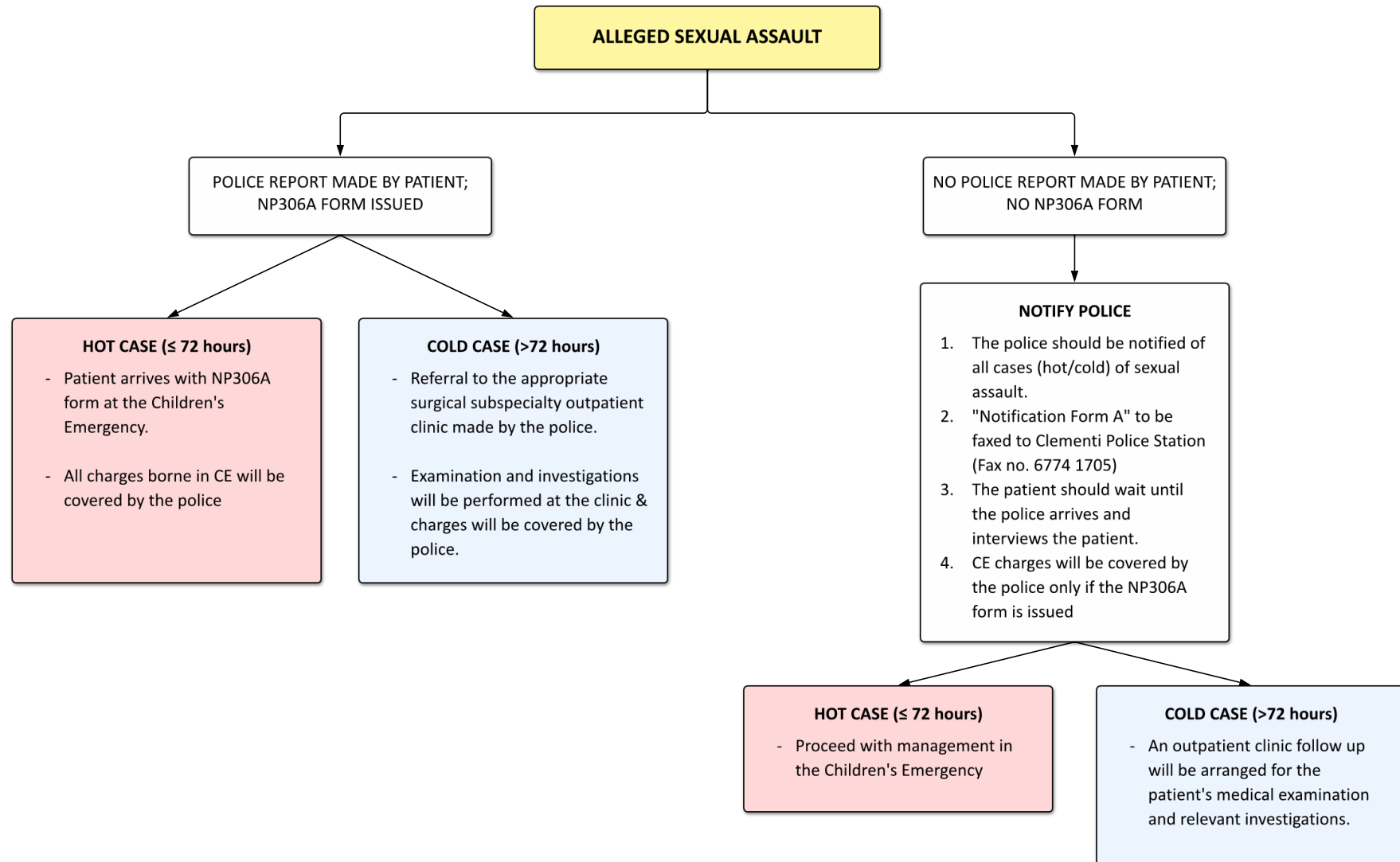
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OVERVIEW OF MANAGEMENT



MANAGEMENT OF ALLEGED SEXUAL ASSAULT (REFERENCE)

		"HOT" CASES		"COLD" CASES
		MALE	FEMALE	MALE/FEMALE
CLINICAL EVALUATION		Refer <u>PAS</u> for physical exam & collection of swab specimens	Refer <u>O&G</u> for gynecological exam & collection of swabs specimens	If police report not made prior, await for arrival of police to interview patient. Non-urgent review at surgical subspecialty outpatient clinic after discharge from CE
POST-EXPOSURE COUNSELLING		<u>Counselled by CE doctors.</u> 1. HIV PEP 2. Screening & empiric treatment of STIs	<u>Counselled by O&G as part of evaluation.</u> 1. HIV PEP 2. Post-Coital Contraception 3. Screening & empiric treatment of STIs	<u>Refer O&G if patient requests for:</u> 1. Contraception > 5 days from encounter 2. UPT+ or Termination of pregnancy (if relevant)
INVESTIGATIONS		<u>STI Screen (NUH Lab)</u> 1. HIV Ag-Ab Screening 2. Syphilis Screen 3. Anti-HBS, HbsAg 4. Anti-HCV 5. Chlamydia trachomatis & Neisseria Gonorrhea (CTNG) DNA PCR (FVU) <u>Baseline Bloods (NUH Lab)</u> 1. FBC, LFT, RP <u>Miscellaneous (HSA)</u> 1. Toxicology, urine 2. Toxicology, blood	<u>STI Screen (NUH Lab)</u> 1. HIV Ag-Ab Screening 2. Syphilis Screen 3. Anti-HBS, HbsAg 4. Anti-HCV 5. Chlamydia trachomatis & Neisseria Gonorrhea (CTNG) DNA PCR (FVU) 6. Trichomonas, Candida, and Bacterial Vaginosis (HVS) <u>Baseline Bloods (NUH Lab)</u> 1. FBC, LFT, RP <u>Miscellaneous (HSA)</u> 1. Toxicology, urine 2. Toxicology, blood	<u>UPT (females)</u> <u>STI Screen</u> To be performed with perineal examination at the outpatient clinic follow up (where NP 306A form will be filled).
MEDICATION		See <u>Medication Table</u>	See <u>Medication Table</u>	As indicated.
DOCUMENTS		MC x 1 week	MC x 1 week	MC (as indicated)
FOLLOW UP	All patients	- PAS x 1 week - MSW x 2 days	- Clinic G x 4 weeks - MSW x 2 days	<u>Outpatient Surgical Subspecialty Clinic for perineal examination</u> - Female: Clinic G - Male: Paeds Surgery - MSW x 2 days Ensure that the police have interviewed the victim prior to discharge.
	If started on HIV PEP/ positive for syphilis	<u>Age ≥ 16 years</u> - BePREP Clinic x 3 – 5 days (call 91862310 if not contacted for TCU) <u>Age < 16 years</u> - Paeds ID x 3 – 5 days	<u>Age ≥ 16 years</u> - MIDC Clinic x 3 – 5 days <u>Age < 16 years</u> - Paeds ID x 3 – 5 days	

FVU: First Void Urine; **HVS:** High Vaginal Swab; **PAS:** Pediatric Surgery

1. LAWS CONCERNING SEXUAL OFFENCES IN SINGAPORE

TERM	DEFINITION
Sexual assault	Sexual contact or behavior that occurs without the explicit consent of the victim.
Age of consent (≥ 14 years)	A minor < 14 years old is deemed unable to communicate consent to sex - Voluntary consent is deemed invalid if the person was intoxicated or under the influence of drugs. - A person cannot give consent if the offender makes a threat to harm them or is in an exploitative relationship with the offender.
Exploitative Relationship (Age < 18 years)	An exploitative relationship will be determined by the court based on certain factors & the circumstances of the case if the victim is a minor below the age of 18. Factors include: - Age of the minor - Age difference between the minor & the accused - The nature of the relationship - The degree of control or influence the accused has over the minor. Examples: - A teacher-student relationship - An attorney-client relationship - A parent-child relationship - A doctor-patient relationship - A custodial officer-detainee relationship
Underage sex (< 16 years)	The legal age for consensual sex is 16 years old. A person is guilty of underage sex if the person whom they had sex with is below 16 years old.
Rape ¹	When a <u>man's</u> penis enters a victim's (male or female) vagina, anus, or mouth without their consent.
Statutory rape	If the victim is below 14 years old at the time of the occurrence, penile penetration will still be considered rape, whether or not the victim consented to the conduct. If both parties are below 14 years or 15 years of age, this still constitutes the offence of statutory rape. However, the police may not initiate charges after taking into account the age of both parties & the circumstances surrounding the alleged act in question.

¹ By this definition, only men can be charged for rape.

2. DETAILED MANAGEMENT

2.1 “HOT” CASES

2.1.1 FEMALE VICTIMS

O&G	- Obtain signed consent for examination. - Perform physical & vaginal examination. - Collects the following specimens: - Clothes - Swabs (<u>Mandatory</u>: Vulval, upper & lower vaginal, endocervical; <u>If indicated</u>: Rectal, buccal etc.) - Perform counselling for STI PEP & post-coital contraception. - Dispense medications for post-coital contraception, STI and HIV prophylaxis to victim. - Put up referral orders for appointment booking: - Adolescent Gynae Clinic x 4 weeks - MSW – Crisis Intervention & Risk Assessment – Alleged Sexual Assault/Underage consensual sex (within 2 days) - If started on HIV PEP or syphilis positive ≥ 16 years: TCU MIDC Clinic x 3 – 5 days < 16 years: TCU Paeds ID Clinic x 3 – 5 days - Issue medical certificate for 1 week
CE NURSE	- Record down: - Victim’s contact details - Contact details of a competent caregiver - Investigation Officer’s (IO) contact number - Victim’s anthropometry (height & weight) - Victim’s Clothes (sealed brown envelope) - Lay out “Trace Evidence” paper (in SA kit) on the floor for victim to undress on; fold & place paper into brown envelope provided after victim has undressed. - Place victim’s clothes into brown envelopes provided and seal envelopes. - Samples - Obtain urine for pregnancy & toxicology (if indicated) - Label each specimen (dry swabs) with victim’s particulars, nature of specimen (where applicable), site, and sign on all security seals with institution logo. - Sexual Assault (SA) Kit - Place all required items for dispatch to HSA in SA kit. - Seal SA kit with signed security seals and “Evidence” sticker on all sides - Inform PSA Duty Supervisor (97261584) to arrange for courier dispatch of SA kit to HSA (During office hours: 9am to 345pm.) SA kit to be kept in Room 11 fridge if not dispatched. - NP306A Form (Pink Form) - Complete witness section on NP306A form once completed by O&G doctor. - Make 1 copy of NP306A Form (for MSW) - Duplicate (carbon copy) of NP306A (for MRO) - Dispatch the specimens & documents (see “Summary of Specimens/Documents to Dispatch”) - Complete Sexual Assault Management Checklist and file in SA Examination Room
CE DOCTOR	- Venipuncture for relevant investigations - Trace O&G doctor’s notes and ensure all changes have been followed through. - Medications prescribed. - Brief history entered into EPIC (especially for Hot Cases – information relevant to clinical decision making re: STI risk, see “PEP Assessment”)

2.1.2 MALE VICTIMS

PAEDS SURG	1. Obtain signed consent for examination. 2. Perform physical & perineal examination. 3. Collects the following specimens: a. Clothes b. Swabs 4. Complete NP306A form & documentation of usage of SA Kit
CE NURSE	1. Record down: a. Victim's contact details b. Contact details of a competent caregiver c. Investigation Officer's (IO) contact number 2. Victim's anthropometry (height & weight) 3. Victim's Clothes (sealed brown envelope) a. Lay out "Trace Evidence" paper (in SA kit) on the floor for victim to undress on; fold & place paper into brown envelope provided after victim has undressed. b. Place victim's clothes into brown envelopes provided and seal envelopes. 4. Samples a. Obtain urine for toxicology (if indicated) b. Label each specimen (dry swabs) with victim's particulars, nature of specimen (where applicable), site, and sign on all security seals with institution logo. 5. Sexual Assault (SA) Kit a. Place all required items for dispatch to HSA in SA kit. b. Seal SA kit with signed security seals and "Evidence" sticker on all sides c. Inform PSA Duty Supervisor (97261584) to arrange for courier dispatch of SA kit to HSA (During office hours: 9am to 345pm.) SA kit to be kept in Room 11 fridge if not dispatched. 6. NP306A Form (Pink Form) a. Complete witness section on NP306A form once completed by Paeds Surg doctor. b. Make 1 copy of NP306A Form (for MSW) c. Duplicate (carbon copy) of NP306A (for MRO) 7. Dispatch the specimens & documents (see "Summary of Specimens/Documents to Dispatch") 8. Complete Sexual Assault Management Checklist and file in SA Examination Room
CE DOCTOR	1. Venipuncture for relevant investigations 2. Perform counselling for screening & empiric treatment for STIs and HIV PEP. 3. Dispense medications for empiric STI treatment and HIV PEP to victim. 4. Brief history entered into EPIC (especially for Hot Cases – information relevant to clinical decision making re: STI risk, see "PEP Assessment") 5. Put up referral orders for appointment booking: a. Paeds Surg x 1 week b. MSW – Crisis Intervention & Risk Assessment – Alleged Sexual Assault/Underage consensual sex (within 2 days) c. If started on HIV PEP or syphilis positive i. ≥ 16 years: TCU BePREP Clinic x 3 – 5 days ii. < 16 years: TCU Paeds ID Clinic x 3 – 5 days 6. Issue medical certificate for 1 week

2.2 “COLD” CASES

CE DOCTOR	1. Take brief history (identify patient safety issues). 2. (Female) UPT 3. (Female) Acute referral to O&G if: a. Patient UPT+ or requests for termination of pregnancy. b. Contraception > 5 days from encounter 4. Consider investigations if patient is symptomatic (e.g., urine culture, FVU for gonorrhea chlamydia PCR) 5. Issue medical certificate for 1 week (as necessary). 6. Ensure police officer has interviewed patient prior to discharge. 7. Follow up: a. TCU Clinic G (female) / Paeds Surg (male) (for detailed examination for NP306A form) b. MSW – Crisis Intervention & Risk Assessment – Alleged Sexual Assault/Underage consensual sex (within 2 days)
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3. IMPORTANT GUIDES/LISTS FOR REFERENCE

3.1 SUMMARY OF SPECIMENS/DOCUMENTS TO DISPATCH

Sexual Assault Kit (HSA)	1. RED-striped Forensic Tamper Evidence Bag a. Dry swabs (black-capped) 2. GREEN-striped Forensic Tamper Evidence Bag (if indicated, for Toxicology) a. 2 blood tubes (green-capped) b. 1 urine container (yellow-capped) 3. Documentation on Usage of Sexual Assault Examination Kit form (yellow NP306C form) 4. Photocopy of 1st page of NP306A pink form
NUH Specimen Bag (NUH Lab)	1. Blood tubes specimen for STI tests 2. Wet swabs/urine samples for chlamydia & gonorrhea
Sealed NUH Envelopes	1. Photocopy NP306A pink form (to MSW) 2. Duplicate (carbon copy) NP306A pink form (to MRO)
Investigation Officer	1. Original NP306A pink form 2. Victim’s clothes (in sealed brown envelope)

3.2 LIST OF INVESTIGATIONS

NO.	INVESTIGATION	
STI SCREENING		
1	HIV Ag-Ab Screening	Blood
2	Syphilis Screen	
3	Hepatitis B Surface Antibody (Anti-HBS)	
4	Hepatitis B Surface Antigen (HbsAg)	
5	Hepatitis C Antibody (Anti-HCV)	
6	Chlamydia Trachomatis & Neisseria Gonorrhoeae (CTNG) DNA PCR <u>Urine</u> ▫ 10 – 15ml of <u>first void urine</u> in a container without preservatives ▫ Patient should NOT have urinated within 2 hours prior to this collection. <u>Endocervical or urethral swab in transport medium</u> ▫ Collect from Molecular Diagnostic Centre (if needed)	Urine / Swab
7	Trichomonas, Candida, and Bacterial Vaginosis (High Vaginal Swab)	Swab
BASELINE BLOODS (for HIV PEP) – “HOT” Cases Only		
1	Full Blood Count (FBC)	Blood
2	Liver Panel (LFT)	
3	Renal Panel Ext (RP)	
MISCELLANEOUS (if indicated)		
1	Toxicology (sent to HSA) ▫ Blood – Lithium-Heparin Tubes (green-capped) x 2 ▫ Urine – Urine container (yellow-capped) x 1 <i>Bundled with SA Kit if collected (Summary of Specimens/Documents to Dispatch)</i>	
2	Urine Pregnancy Test (UPT)	Urine

3.3 MEDICATION TABLE

All drugs to be ordered under “Dispo” activity tab except for IM ceftriaxone – to be ordered under “Orders” activity tab for administration in CE.

DRUG	ROUTE	DOSE	REMARKS
PEP for STI (All)			
Azithromycin	PO	≤ 45kg: <u>20 mg/kg</u> (max. 1g) ONCE > 45kg: 1g ONCE	Treatment for chlamydia & gonorrhea.
Ceftriaxone	IM	Adolescent: 250 mg ONCE (Order on MAR for administration in CE)	
Metronidazole	PO	Adolescent: 2g ONCE	Treatment for trichomonas
HIV PEP (Truvada & Raltegravir)			
Truvada (Emtricitabine 200mg, Tenofovir 300mg)	PO	< 35kg: Not recommended. ≥ 35kg: 1 tab OM for 5 days	HIV post-exposure prophylaxis offered if ≤ 72 hours after possible HIV exposure. Prescription made for 5 days with an early Infectious Disease follow up. Total course is 28 days. 1. Baseline bloods: FBC, LFT, RP 2. Advise on S/E: Headache, rash, GI symptoms
Raltegravir	PO	400mg BD for 5 days	
Post-coital Prophylaxis (1 st line: ellaOne®)			
ellaOne® (Ulipristal acetate) (up to <u>5 days</u> after UPSI [^])	PO	30mg STAT	Offer to post-menarche patients within window period. Contraindications for (ellaOne®) - Liver enzyme inducers (e.g., anti-epileptics, rifampicin etc.) - Low gastric pH - Glucocorticoids for asthma
OR			
Postinor® (Levonorgestrel) (up to <u>3 days</u> after UPSI)	PO	1.5mg STAT 3mg (double dose) if: BMI >26m/kg ² , weight >70kg, or on liver enzyme inducers	

[^]UPSI: Unprotected Sexual Intercourse

3.4 GUIDANCE FOR POST-EXPOSURE PROPHYLAXIS AFTER SEXUAL EXPOSURE (PEPSE)

3.4.1 BASHH GUIDELINES PEPSE PRESCRIBING RECOMMENDATIONS²

	Source HIV Status			
	HIV-positive		Unknown HIV status	
	HIV VL unknown/ detectable (>200 copies/mL)	HIV VL undetectable (<200 copies/mL)	From high prevalence country/ risk group (e.g. MSM)	From low prevalence country/ group
Receptive anal sex	Recommend	Not recommended (Provided source has confirmed <200 copies/mL for >six months)	Recommend	Not recommended
Insertive anal sex	Recommend	Not recommended	Consider	Not recommended
Receptive vaginal sex	Recommend	Not recommended	Consider	Not recommended
Insertive vaginal sex	Consider ^A	Not recommended	Consider	Not recommended
Fellatio with ejaculation	Not recommended	Not recommended	Not recommended	Not recommended
Fellatio w/o ejaculation	Not recommended	Not recommended	Not recommended	Not recommended
Splash of semen into eye	Not recommended	Not recommended	Not recommended	Not recommended
Cunnilingus	Not recommended	Not recommended	Not recommended	Not recommended
Sharing of injecting equipment	Not recommended	Not recommended	Consider	Not recommended
Human bite ^B	Recommend	Not recommended	Not recommended	Not recommended
Needlestick from a discarded needle in the community	Not recommended	Not recommended	Not recommended	Not recommended

^A Co-factors that influence the likelihood of transmission should be considered.

^B A bite is assumed to constitute breakage of the skin with passage of blood.

² Cresswell F, Waters L, Briggs E, et al. UK guideline for the use of HIV Post-Exposure Prophylaxis Following Sexual Exposure, 2015. Int J STD AIDS. 2016;27(9):713-738. doi:10.1177/0956462416641813

3.4.2 HIV PREVALENCE IN SINGAPORE

- High prevalence is > 1%.
- HIV prevalence in Singapore³
 - Adults (15 – 49 years) 0.2%
 - Young men/women <0.1%
 - MSM 2.2%
 - Prisoners 1.1%

3.4.3 FACTORS INCREASING THE RISK OF HIV TRANSMISSION

1. **A high plasma HIV viral load (VL) in the source** – with each log₁₀ increase in plasma HIV RNA, the per-act risk of transmission is increased 2.9-fold [95% CI 2.2 – 3.8]. This may be particularly relevant during primary HIV infection.
2. **Breaches in the mucosal barrier** such as mouth or genital ulcer disease and anal or vaginal trauma following sexual assault or first intercourse.
3. **Menstruation or other bleeding** – theoretical risk only
4. **Sexually transmitted infections** in HIV positive individuals not on ART or HIV-negative individuals with genital ulcer disease.
5. **Ejaculation** – Among a community cohort of men who have sex with men (MSM), the risk of HIV acquisition per episode of unprotected receptive anal intercourse with and without ejaculation was estimated to be 1.43% [95% CI 0.48 – 2.85] and 0.65% [95% CI 0.15 – 1.53], respectively.
6. **Non-circumcision** – circumcision has been shown to significantly reduce HIV acquisition among heterosexual men in high prevalence countries. In 2008, a meta-analysis of observational studies in MSM suggests circumcision has little impact upon HIV acquisition. However, since then the risk of HIV acquisition per episode of unprotective insertive anal intercourse in circumcised men was estimated to be 0.11% [95% CI 0.02 – 0.24] versus 0.62% [95% CI 0.07 – 1.68] in uncircumcised men in a community cohort of MSM in Australia
7. **Discordant HIV VL in the genital tract** – in general, the genital tract VL is undetectable when the plasma VL is undetectable. When this is not the case, the VL in the genital tract is usually low.

³ <https://www.unaids.org/en/regionscountries/countries/singapore> (2021)

3.4.4 COUNSELLING POINTS

1. What is Post-Exposure Prophylaxis (PEP)?
 - a. PEP means taking medicine to prevent HIV after a possible exposure.
 - b. PEP should be used only in emergency situations and must be started within 72 hours after a recent possible exposure to HIV.
 - c. If taken within 72 hours after possible exposure, PEP is highly effective in preventing HIV – though not 100%. It is important to take the medications daily, as prescribed.
2. Side effects of PEP.
 - a. PEP is safe but may cause minor side effects like nausea, watery stools, or headaches. In almost all cases, these side effects can be treated and are not life threatening. Patients are advised to return immediately if symptoms are worsening.
 - b. Patients with skin rash or flu-like symptoms after starting PEP should return for an urgent review to exclude an HIV seroconversion illness.
 - c. Blood tests will need to be performed before the drugs can be prescribed.
3. You may be given an appointment with an infectious disease specialist. Please do not cancel or postpone this appointment as it can have serious health implications.
4. Duration of PEP.
 - a. The full course of PEP is 28-days long. You may be given a prescription for 5 days' worth of two medicines (This will cost around \$42 per day).
 - b. After reviewing the results of the initial blood tests, the specialist may advise completing the 28-day course (cost more than \$1000), or stopping HIV PEP (if not medically indicated, or risks outweigh the benefits).
 - c. Avoid missing doses of PEP as drug levels may drop below the minimum required for protection.
5. A follow up with a medical social worker (MSW) in the next 48 hours will be arranged.

3.4.5 POST-EXPOSURE PROPHYLAXIS (PEP) ASSESSMENT

Characteristics of Exposure			
☐ Condom	☐ Used ☐ Not used ☐ Broke ☐ Fell off		
☐ Anal sex	☐ Receptive	☐ Ejaculation	☐ Withdrawal
	☐ Penetrative	☐ Circumcised	☐ Uncircumcised
☐ Vaginal sex	☐ Receptive		
	☐ Penetrative	☐ Circumcised	☐ Uncircumcised
☐ Fellatio (giving)	☐ Ejaculation	☐ Withdrawal	
☐ Semen splashing into eye			
☐ Human bite			
☐ Sharing injecting equipment			
Characteristics of Source			
Source Details	☐ Male	☐ Female	No. of Partners: _____
Source Risk Factors	☐ MSM	☐ Injecting drug use	☐ High prevalence country: _____
Source HIV Status	☐ Positive	☐ No ART/unknown	☐ On ART
	☐ Suspected		
	☐ Unknown		
Patient's PEP Assessment			
Medication history			
Drug allergies			
Comorbidities			

4. REFERRAL ORDERS

4.1 MSW REFERRAL

1. Under “Dispo” activity tab, order “**Referral to Medical Social Worker (aka MSW) – NUH / AH**”

Order Search

MSW

Browse Preference List Facility List

Panels (No results found)

After Visit Drugs (No results found)

After Visit Procedures

Name	Type	Pref List	Px Code	Order Comments	For...
Referral to Medical Social Worker (aka MSW) - NUH / AH	Referral	NGEMR...	2102018		
Referral to Medical Social Worker (aka MSW) - OneSafe (aka M...	Referral	NGEMR...	2102018		

2. Under Reason for Referral, select “**Crisis Intervention & Risk Assessment – Alleged Sexual Assault**” or “**Crisis Intervention & Risk Assessment – Underage consensual sex**”.
3. Under comments, include patient’s contact details, caregiver’s contact details.

Referral to Medical Social Worker (aka MSW) - NUH / AH

Accept Cancel

Scheduling Instructions

STAT Direct Access (<=2 weeks) Early (<=4 weeks) Routine (<=8 weeks) Open Date (patient may go for subsidized visit)

Referral to (Person) :

Reason for Referral

Referral Type? :

Comments:

Title	Number
Crisis Intervention & Risk Assessment - Alleged Sexual Assault	50
Crisis Intervention & Risk Assessment - Alleged Vulnerable Adult Abuse/Neglect	52
Crisis Intervention & Risk Assessment - Domestic Violence	53
Crisis Intervention & Risk Assessment - Interpersonal Violence	49
Crisis Intervention & Risk Assessment - Single Mother	57
Crisis Intervention & Risk Assessment - Suicide & Self-harm	56
Crisis Intervention & Risk Assessment - Underage consensual sex	54

Next Required

4.2 MULTIDISCIPLINARY INFECTIOUS DISEASES CLINIC (MIDC) REFERRAL (FEMALES)

1. Under “Dispo” activity tab, order “**Referral to Infectious Disease – OneSafe**”.
2. Under comments, include **patient’s contact details**, **caregiver’s contact details**.

Referral to Infectious Disease - OneSafe
Accept
Cancel

Reason for Referral
HIV
Testing and Treatment for STIs and HIV
TB
Travel Medicine
General ID

Reason
Newly Diagnosed HIV
Chronic HIV
HIV in Pregnancy
Post-exposure Prophylaxis
Pre-exposure Prophylaxis

Please contact the following for referral:bpc@nuhs.edu.sg or +65 9186 2310
OneSafe Workflow - appointment within 5 days

Scheduling Instructions ⓘ
Fast Track (<=1 week)
Direct Access (<=2 weeks)
Early (<=4 weeks)
Routine (<=8 weeks)
Open Date (patient may go for subsidized visit)

OPTIONAL - Patient's Preferred Institution
OneSafe Workflow - appointment within 5 days

Industrial Accident?
Yes
No
Unknown/Unsure

Referral Type? :
Subsidized referral if eligible
Non-subsidized referral

Process Instructions:
Immediate referral to EMD
1. Any patient who is acutely unwell
Fast Track (Within 1 week)
1. Newly diagnosed HIV, Chronic HIV and HIV in Pregnancy
A. Contact NUH MIDC @ nuh_midc@nuhs.edu.sg or +6772 7800 ext. 2 for the referral

Comments:
OneSafe Workflow - appointment within 5 days

Next Required
Accept
Cancel

4.3 BEPREP CLINIC REFERRAL (MALES)

1. Under “Dispo” activity tab, order “**Referral to Infectious Disease (ID) – NUHS**”.

After Visit Procedures						
	Name	Type	Pref List	Px Code	Order Comments	For...
	Referral to Infectious Disease (ID) (Dengue) - NUHS	Referral	NGEMR...	210904		
	Referral to Infectious Disease (ID) - NUHS	Referral	NGEMR...	210904		
	REFERRAL TO INFECTIOUS DISEASE - NON-EPIC	Referral	NGEMR...	210121		
	Referral to Infectious Disease - OneSafe	Referral	NGEMR...	210904		

2. Reason for Referral: **HIV**.
3. Reason: **Post-exposure Prophylaxis**.
4. Enter “**BePREP Clinic, sexual assault workflow, within 5 days**” for the next 2 text boxes.
5. Under comments, include **patient’s contact details, caregiver’s contact details**.

Referral to Infectious Disease (ID) - NUHS

AcceptCancel

Reason for Referral

HIVTesting and Treatment for STIs and HIVTBTBTravel MedicineGeneral ID

Reason

Newly Diagnosed HIVChronic HIVHIV in PregnancyPost-exposure ProphylaxisPre-exposure Prophylaxis

Please contact the following for referral:

bpc@nuhs.edu.sg or +65 9186 2310

BePREP Clinic, sexual assault workflow, within 5 days

Scheduling Instructions

Fast Track (<=1 week)Direct Access (<=2 weeks)Early (<=4 weeks)Routine (<=8 weeks)

Open Date (patient may go for subsidized visit)

OPTIONAL - Patient's Preferred Institution

BePREP Clinic, sexual assault workflow, within 5 days

Industrial Accident?

YesNoUnknown/Unsure

Referral Type? :

Subsidized referral if eligibleNon-subsidized referral

Process Instructions:

Immediate referral to EMD
1. Any patient who is acutely unwell

Fast Track (Within 1 week)
1. Newly diagnosed HIV, Chronic HIV and HIV in Pregnancy
A. Contact NUH MIDC @ nuh_midc@nuhs.edu.sg or +6772 7800 ext. 2 for the referral

Comments:

ED/EDTU After Visit Referral

Dear Colleague,

Next Required

AcceptCancel